Machine Translated by Google

Ministry of Public Health, Hygiene and Social Welfare
National Institute of Public Health
Public Health Emergency Operations Center
PRESIDENTIAL EBOLA TASK FORCE 17
MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°147 /MVE-BDBV/08/10/2026

Report date: October 8, 2026 Publication date: October 9, 2026

HIGHLIGHTS (last 24 hours)

- **Fifty-eight (58) new confirmed cases of Ebola Virus Disease (EVD), including 14 deaths**
  Community transmission of COVID-19 cases has been reported. These cases originated from the provinces of North Kivu (30 cases), Ituri (20 cases), Tshopo (4), Haut-Uele (3), and Bas-Uele (1). In addition, 19 deaths were recorded in COVID-19 treatment center
- **A total of 390 new samples were analyzed; 58 of these samples tested positive for the virus Bundibugyo, or 14.9% positivity.**

ACCUMULATIVE CAS	ACCUMULATIVE DEATH CONFIRMED	LETHALITY	PATIENTS IN ISOLATION/CTE	ACCUMULATIVE HEALED	TRACKING RATES CONTACTS (Of the Day)
8845	4 275	48,3 %	950	2 320	83,5%

Affected provinces and affected health zones

7 Provinces affected
Uele, Ituri, North Kivu, South Kivu, Tshopo, Lower Uele and Sud Ubangi

65 out of 167 health zones affected (38,9%)

Summary of key points from the last 24 hours

For the day of October 8, 2026, 58 new confirmed cases of Ebola Virus Disease (EVD) including 14 community deaths were reported in 5 provinces: North Kivu (30 cases), Ituri (20 cases), Tshopo (4 cases), Haut-Uélé (3 cases) and Bas-Uélé (1 case).

In addition, 19 deaths were recorded in the CTEs (9 in Ituri, 7 in North Kivu and in Tshopo (2) in Haut Uele (1), which brings the total number of confirmed deaths for the period concerned to 33.

On the other hand, 28 patients (14 in Ituri, 11 in North Kivu and 3 in Haut-Uélé) were declared cured after obtaining two negative results in the control test for Ebola virus disease.

As of October 8, 2026, the DRC has recorded **8,845** confirmed cases and **4,275** deaths, representing a case fatality rate of **48.3%.** Ituri remains the epicenter of the epidemic, accounting for **74.2** % of cumulative confirmed cases and **34.35%** of new confirmed cases reported in the last 24 hours.

With the addition of the Dakwa health zone in Bas Uélé, 65 out of 167 health zones (38.9%), spread across seven provinces, have been affected since the start of the epidemic.

Machine Translated by Google

Table 1. Distribution of confirmed cases and deaths by affected province, as of October 8, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	20	6562	3 036	46,3%	28/36 (77,8 %)
North Kivu	30	1841	1 054	57,3%	17/34 (50,0 %)
Haut-Uélé	3	371	157	42,3%	7/13 (53,8 %)
Boss	4	55	22	40,0%	7/23 (30,4 %)
South Kivu	0	3	1	33,3%	1/34 (2,9 %)
Bas-Uélé	1	11	4	36,4%	4/11 (36,4%)
Sud Ubangi	0	2	1	50,0%	1/16 (6,3%)
Total	**58**	**8 845**	**4 275**	**48,3%**	**65/167 (38,9%)**

Table 2. Distribution of confirmed cases and deaths by province and zone as of October 8, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Cas (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	**6562**	**3036**	**46,3%**	**20**	**2**	**9**	**11**
Give	12	1	8,3%				0
Understand	8	6	75,0%				0
Ariwara	8	2	25,0%				0
Aungba	12	5	41,7%				0
Bamboo	213	47	22,1%	3			0
have	2	2	100,0%				0
Bunia	1798	599	33,3%	1		3	3
Ladies	40	9	22,5%				0
Drodro	15	7	46,7%				0
Fireworks	84	31	36,9%				0
Gethy	9	5	55,6%				0
Flounder	2	0	0,0%				0
Kilo	38	12	31,6%				0
Team	176	107	60,8%				0
Liter	295	149	50,5%			1	1
Logo	14	7	50,0%				0
Fight	35	9	25,7%				0
distributed	4	1	25,0%				0
Washing	41	17	41,5%				0
Darkness	217	108	49,8%	8	1	2	3
Mangala	366	157	42,9%				0
Mongbwalu	710	338	47,6%			1	1
Nanny	317	178	56,2%	1		1	1
Nizi	802	289	36,0%	3			0
Legumes	133	39	29,3%	1			0
Jungle	14	5	35,7%				0
Rwampara	1121	388	34,6%	3	1	1	2

Machine Translated by Google

Tchomia	76	25	32,9%				0
A valves	THAT	493	THAT				0
North Kivu	**1841**	**1054**	**57,3%**	**30**	**11**	**7**	**18**
Me	430	283	65,8%	10	5	0	5
Farm	23	7	30,4%				0
Butembo	293	185	63,1%	4	1	2	3
Ten	1	0	0,0%				0
Kalunguta	66	24	36,4%	3	1	1	2
Cut	619	379	61,2%	6	2	3	5
Source	3	2	66,7%				0
Kyondo	37	20	54,1%				0
Lubero	23	11	47,8%				0
Be anxious.	55	21	38,2%	3	1	0	1
Alimbong	5	3	60,0%				0
Manguredjipa	13	5	38,5%				0
Masers are	36	11	30,6%				0
The museums	141	69	48,9%	1	0	0	0
Mutwanga	20	4	20,0%				0
Oicha	37	18	48,6%	1	1	0	1
Vuh	39	12	30,8%	2		1	1
Upper Uele	**371**	**157**	**42,3%**	**3**	**0**	**1**	**1**
Government Mangbetu	44	19	43,2%				0
Dumb	1	1	100,0%				0
Gombari	3	1	33,3%				0
Mathematics	102	44	43,1%				0
Lining	97	47	48,5%	2	0	1	1
Beans	4	2	50,0%				0
Ordinary	120	43	35,8%	1	0	0	0
Boss	**55**	**22**	**40,0%**	4	1	2	3
Bafwasende	7	1	14,3%				0
Kabondo	5	1	20,0%				0
Lubunga	1	0	0,0%				0
Maquis - Kisangani	29	12	41,4%	2		1	1
Mangobo	10	7	70,0%	2	1	1	2
Boss	2	0	0,0%				0
Wanie - Arugula	1	1	100,0%				0
South Kivu	**3**	**1**	**33,3%**	0	0	0	0
Miti -Murhesa	3	1	33,3%				0
Bas Uélé	**11**	**4**	**36,4%**	1	0	0	0
Blind	1	1	100,0%				0
Viadana	6	1	16,7%				0
Claim	1	0	0,0%	1	0	0	0
Walk	3	2	66,7%				0
Sud Ubangi	**2**	**1**	**50,0%**	0	0	0	0
Hair	2	1	50,0%				0
Total	**8845**	**4275**	**48,3%**	**58**	**14**	**19**	**33**

Machine Translated by Google

*These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.
NA : Not Applicable.

Table 3. Status of alerts notified by province for October 8, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	139	6	145	45	6	94	0	51	15
Bas Uélé	133	2	135	16	2	117	0	18	0
Ituri	ND	ND	ND	ND	ND	ND	ND	ND	ND
North Kivu	1 235	62	1297 129		62	1 106	0	191	99
Sud Ubangi	ND	ND	ND	ND	ND	ND	ND	ND	ND
Boss	143	2	145	10	0	133	2	10	0
Total General	**1 650**	**72**	**1 722 200**		**70**	**1 452**	**2**	**270**	**114**

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

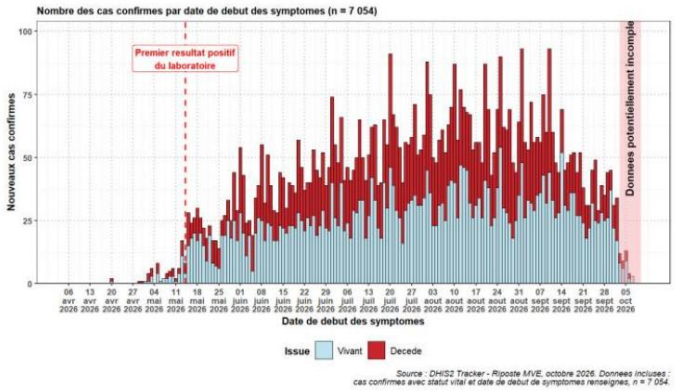
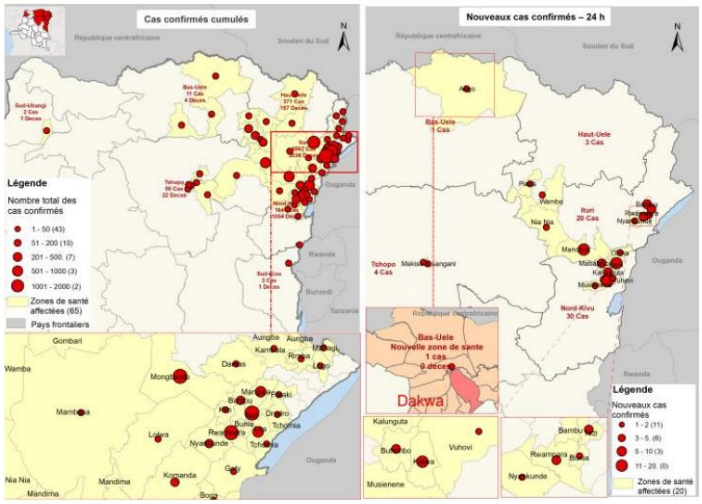


Figure 2. Spatial distribution of confirmed cases (24 hours and cumulative) by health zone.



1. COORDINATION

1.1. Main Actions

- Support for response activities in the affected provinces through the technical pillars.

2. RESPONSE ACTIONS BY PILLAR

2.1. Epidemiological surveillance

2.1.1. Main Actions

- As of October 8, 2026, 1,722 alerts had been reported in four provinces. All were verified (100.0%), of which 270 (15.6%) were validated as suspected cases and 1,452 (84.3%) invalidated. All 270 suspected cases were investigated (100.0%). Of these, 114 (42.2%) were transferred to CT/CTE.

Machine Translated by Google

• Of the 29,140 contacts being monitored, 24,337 were seen in the last 24 hours, representing a follow-up rate of 83.5%, up from the previous day (80.0%) but below the required threshold of 95.0%. By province, this rate was 95.5% (612/641) in Tshopo, 92.5% (172/186) in Bas-Uélé, 85.5% (1,779/2,081) in Haut-Uélé, 83.9% (9,038/10,772) in Ituri, and 82.4% (12,736/15,460) in North Kivu; only Tshopo reached the 95% threshold.

2.2. Update on checkpoint and point of entry (PoE/PoC) activities

2.2.1. Main actions

• Six (6) alerts were notified to the PoE/PoCs listed in Table 4 on October 8: four (4) in Ituri and two (2) in North Kivu, none in Tshopo; the 3 confirmed suspected cases were all investigated within the allotted timeframe. In Ituri, the alert from the Iga Barrière PoC (Nizi Health Zone) was invalidated after secondary screening, those from Bunia Airport and the Mudzi Bala PoC (Bunia Health Zone) were transferred to the Elikia and Bunia Ebola Treatment Centers respectively, and a body found at the DGRPI Nizi PoC was swabed by the EDS team (result pending).

Table 4. Status of key indicators for PoE/PoC by province, as of October 8, 2026

Indicators	Ituri	North Kivu	Boss	Global
Report completeness	98,4% (60/61)	95,2% (20/21)	100,0% (18/18)	98,0% (98/100)
% of functional strategic PoE/PoC	100%	100%	100%	100%
Number of people who have passed PoE/PoC	152 060	91 930	18 484	262 474
% of travelers screened	98,2%	97,5%	98,6%	98,0%
% of travelers who washed their hands	97,7%	97,5%	98,5%	97,7%
% of travelers made aware	99,6%	97,5%	100,0%	98,9%
Number of alerts notified	4	2	0	6
Number of validated live alerts (suspected cases)	2	1	0	3
Number of bodies recovered today	1	1	0	2
% of suspected cases transferred to CTE/CT	100,0%	100,0%	THAT	100,0%
% of lifeless bodies swabed/secured	100,0%	100,0%	THAT	100,0%
% of suspected cases investigated within 4 h	100,0%	100,0%	THAT	100,0%

2.2.2. Challenges

In Tshopo, 1.4% of travelers (263/18,484) refused screening and 1.5% (275/18,484) refused handwashing, amid rumors that "the water contains the virus" or "the disease does not exist." In South Kivu, reporting completeness is only 55.6% (5/9). In Bas-Uélé, only 19 Points of Control (PoCs) out of 41 notification sites are functional (46.3%), no Points of Evaluation (PoEs) are strengthened (0/11), reporting completeness is 88.9% (16/18), and the National Humanitarian Action Program (PNHF) services are not operational.

2.3. Laboratory

2.3.1. Main actions

• **Ituri: 20 new positive results** (18 living and 2 dead) out of 139 samples received and analyzed **(positivity: 14.4%).**

• **North Kivu: 30 new positive results** (19 living and 11 dead) out of 180 samples received and analyzed **(positivity: 16.7%).**

Machine Translated by Google

- **Haut-Uélé: 3 new positive results** (2 living and 1 death) **out of 24 samples received and analyzed (positivity: 12.5%).**

- **Tshopo: 4 new positive results (3 blood and 1) out of 14 samples analyzed (positivity: 28.6%).** **receipts** **And**

- **Bas Uélé: 1 positive result** (1 blood test) out of 33 received and analyzed **(positivity: 3.0%).**

2.4. Infection Prevention and Control (IPC/EDS)

2.4.1. Main actions

- **Ituri:** 12 out of 23 rings were opened (52.2%), 17 out of 28 households were decontaminated (60.7%), and all 7 health facilities were decontaminated (100%); no households, social economy enterprises, or schools received kits. Of 99 blood and sanitation alerts (including 8 postponements from the previous day), 57 blood and sanitation tests were carried out (57.6%) and 62 bodies were swabbed, with 32 failures (32.3%) due to refusals in Bambu, Mambasa, Mangala, Fataki, and Mandima; 10 postponements remained at the end of the day.

- **North Kivu:** 12 out of 25 rings were opened (48.0%), with a significant gap in Katwa; 19 out of 25 health centers (76.0%) were decontaminated, and 14 out of 40 households (35.0%) received kits; none of the decontaminated households were provided with kits; 21 infection prevention and control (IPC) assessments (scorecard) were conducted in 21 health centers with an average score of 33%. Out of 58 emergency health alerts, 56 were carried out and 58 bodies were swabbed without failure, with 51 postponed to the following day; the distribution of IPC/WASH kits in 300 schools was launched with the support of CAUB/UNICEF, and the training of 14 community health teams began in Vuhovi, Kyondo, and Beni.

- **Haut-Uélé:** 3 ESS (Pawa CTE, Bachenze CS, and Babonde CS) and 5 households were decontaminated in the Pawa health zone; 4 ESS were assessed (3 in Isiro and 1 in Boma Mangbetu); and 55 service providers were briefed (26 in Pawa, 14 in Wamba, 13 in Isiro, and 2 in Boma Mangbetu). The family of a case in the Bachenze health zone refused household decontamination. Of 4 DHS alerts, 4 DHS were carried out (100%), and 2 bodies were swabned, the other 2 having already been collected at the CT.

- **Tshopo.** 13 out of 13 COVID-19 treatment centers (DTCs) were monitored in 4 health zones, with 137 service providers being briefed on standard precautions, triage, and screening; the household of a confirmed case was decontaminated in Mangobo (7 rooms and one latrine, with the incineration of 2 mattresses), and 2 schools were assessed (EP La Colombe and EP Maendeleo). Since the beginning, 42 out of 42 DTCs (100%) and 32 out of 35 households (91.4%) that hosted a confirmed case have been decontaminated within 48 hours, and 53 out of 103 DTCs (51.5%) have received infection prevention and control (IPC) kits. Regarding the emergency response teams (EDS), out of 4 reports and 3 alerts from the Emergency Treatment Center (ETC), 3 bodies were swabed.

2.4.2. Challenges

2.4.3.

In Ituri, the opening of the rings remains insufficient (52.2%), there are insufficient teams in Komanda, Mangala, and Mandima, mobility is lacking in Mambasa and Mandima, and refusals of EDS (Emergency Health Services) persist. In North Kivu, the gap in ring opening in Katwa, the low rate of household decontamination (14/40; 35.0%), and the lack of distribution of kits to decontaminated households persist.

2.4.4. Holistic care

2.4.5. Main actions

In Ituri, 58 new admissions were recorded, compared to 102 discharges, including 14 recoveries. At the end of the day, 316 patients remained in isolation, representing a bed occupancy rate of 31.1% (316/1,015), including 30 critically ill patients. 310 out of 334 patients (92.8%) received a hot meal, and all children under the LNPE/SLM program received care (12/12 at the CTE, 6/6 at the daycare center, and 49/49 in the community). The province's total number of recoveries has reached 1,850.

- **In North Kivu,** 100 new admissions (15 confirmed and 85 suspected) were recorded, compared to 96 discharges (78 non-cases, 11 recoveries, and 7 deaths). A total of 523 patients remained in isolation at the end of the day, representing a bed occupancy rate of 91.8% (325 patients in CTE/CT beds out of 354 beds), including 3 critically ill patients; 149 patients are being treated in ESS facilities that are not CTE/CTs, and 290 patients, 169 caregivers, and 408 people with disabilities received hot meals.

Machine Translated by Google

- **In Haut-Uélé,** 12 new admissions (4 confirmed, including 3 PPL, and 8 suspected) were recorded, compared to 17 discharges (11 non-cases, 3 recoveries, and 3 deaths). A total of 71 patients remained in isolation at the end of the day, representing an overall bed occupancy rate of 51.4% (71/138); the standardized Ebola Treatment Center (ETC) in Isiro was officially handed over by the WHO to the care team, and 49 patients, 69 caregivers, and 69 PPL received hot meals.

- **In Tshopo,** 3 new admissions (1 confirmed and 2 suspected) were recorded, compared to 4 discharges (2 non-cases and 2 deaths). A total of 14 patients remained in isolation at the end of the day, representing an overall bed occupancy rate of 45.2% (14/31); since the beginning, 424 people have received the experimental treatment, including 37 confirmed cases, and 387 have benefited from post-exposure prophylaxis.

- **In Bas-Uélé,** 18 new admissions (the confirmed case from the Dakwa health zone, isolated at the Dakwa General Referral Hospital, and 17 suspected cases) were recorded with no discharges; in total, 2 patients remained in isolation at the end of the day in non-standard transit centers (Bondo, Viadana, Buta, Dakwa and Ganga), the province not yet having any Ebola treatment centers.

2.4.6. Challenges

- **In Ituri,** the low capacity of some CTE/CTs (Mangala, Kilo, Mandima, Biakato and Lolwa), the care of 30 critical patients and the lack of food for 24 patients in Mandima remain worrying.

- **In North Kivu,** the saturation of suspected cases (159/165; 96.4%) and the management of 149 patients (31.4%) outside of CTE/CT remain a concern.

- **In Haut-Uélé,** the escape of 2 confirmed cases from the Pawa CTE, inconsistencies in care data and the absence of a hearse at the Boma Mangbetu CTE remain challenges to be solved.

- **In Tshopo,** the demotivation of 12 out of 29 ambulance workers (41.4%) undermines the transfer of patients.

- **In Bas-Uélé,** there is a lack of a standardized CTE (Emergency Medical Treatment Center), which is still under construction, and of an ambulance for transfers. Managing patients remains a major operational challenge.

2.5. Vaccination 2.5.1.

Main actions

- **In Bas-Uéléle,** pre-registration of PPL and TPL (religious leaders, RECO, traditional healers, motorcyclists and hunters) continued, with the provincial total reaching 1,500 people vaccinated out of a target of 13,202 (11.4%), mainly in Buta (620; 21.6%), Ganga (273; 20.5%) and Viadana (243; 9.8%); the provincial stock of Ervebo vaccine is exhausted (0 doses).

- **In Haut-Uélé,** the vaccination of PPL and health workers against Ebola was launched in the Mayogo health district by the Provincial Governor, in the presence of the Incident Manager.

- **In Ituri,** the data collection tools for the study on the impact of the Ebola epidemic on The use of sexual and reproductive health services was pre-tested.

2.5.2. Challenges

- **In Ituri,** vaccination of PPL and the study of cross-immunity with the BDBV strain remain suspended pending the supply of Ervebo vaccine by the national PEV.

- **In Bas-Uélé,** the implementation of the UCCs is not effective and the micro-plan has not been validated, as well as the Vaccination funds are still not available.

Machine Translated by Google

2.6. Continuity of Care

2.6.1. Main actions

- **In Tshopo,** the evaluation of the Tshopo 2 and Salama health centers revealed the absence of an effective triage area in both health centers; at the Bondeko health center, 14 out of 14 staff (100%) are vaccinated, 4 out of 16 expected prenatal care appointments (25.0%) were missed and the 16 expected pregnant women kept their prenatal care appointments; the October PCSSE work plan and the recovery plan for the City health center were finalized, and the follow-up of PPLs at the CTM and CTE as well as that of recovered patients with comorbidities at the Makiso-Kisangani General Referral Hospital continued.

- **In North Kivu,** a briefing on the continuity of essential care services in the context of Ebola was organised in the Beni health zone and the Oicha general referral hospital was assessed.

- **In Bas-Uélé,** the evaluation of the Viadana General Referral Hospital shows a decrease in the use of services between May (7,009) and September 2026 (4,874), a bed occupancy rate of 20% in September for 70% expected, the absence of a triage and protection zone for PPL, and 16 PPL vaccinated out of 16.

2.7. Risk Communication and Community Engagement (RCCE)

2.7.1. Main actions

- **In Ituri,** 21,666 home visits reached 55,997 people; 635 educational talks reached 9,859 people, 47 mass awareness campaigns reached 4,928 people, 99 community debates reached 1,469 people, and 7 advocacy sessions involved 15 authorities and leaders; 12 public service announcements were broadcast on 36 radio stations, and 9 programs were organized. A total of 470 community alerts, including 135 fatalities, were reported; 43 out of 79 feedback requests (54.4%) and 4 out of 6 incidents (66.7%) were resolved within 48 hours, and 20 motorcyclists from Bunia and Rwampara were briefed for their involvement in the Community Health Centre (CHC) and alert reporting. The completeness rate of CREC reports is only 36% (10/28 health zones).

- **In Bas-Uélé,** 158 home visits reached 553 people in Ganga and Viadana, and 2 mass awareness sessions reached 672 people; 12 community alerts were reported, all 3 feedback requests received were resolved (100%), and advocacy was conducted with the chief of Kipate for the community reintegration of the Guér (a term used to refer to people who have been in the Guér region).

- **In North Kivu,** 877 people were hired out of "475 expected," representing a reported proportion of 18.5%. The denominator appears to have been misinterpreted. To obtain 18.5%, it should be closer to 4,740. The exact target needs to be confirmed in the provincial report.

- **In Haut-Uélé,** 15 educational talks, 3 mass awareness sessions, and 122 home visits reached 2,712 people in Isiro, Wamba, and Watsa, and 42 community health workers and community health center members were trained; 54 community alerts, including 6 deaths, were reported in 5 health zones, and none of the 13 feedback requests received have yet been resolved. Vaccination of people living with HIV (PLHIV) against Ebola virus disease (EVD) was launched by the Provincial Governor in the presence of the Incident Manager.

2.8. Mental Health and Psychosocial Support (MHPSS)

2.8.1. Main actions

- **North Kivu,** 23 out of 25 confirmed cases (92.0%), 123 confirmed cases under follow-up, 75 new suspected cases and 376 suspected cases under follow-up benefited from SMSPS interventions; the teams accompanied 66 households for public health actions and 23 out of 25 separated and orphaned children households (92.0%), supported 30 PPLs, reintegrated 9 recovered individuals, followed up 19 out of 19 recovered individuals in the community (100%), 6 out of 6 children in daycare and 31 out of 35 children in the community (88.6%), carried out psychoeducation for 344 people (107 men, 166 women and 71 children) with the support of UNICEF and announced 85 results, including 26 positive ones; 7 out of 7 health zones that notified are covered (100%).

- **In Ituri,** 16 out of the last 29 confirmed cases (55.2%), 83 confirmed cases under follow-up out of 118 (70.3%), 18 new suspected cases out of 51 (35.3%), and 95 suspected cases under follow-up out of 128 (74.2%) benefited from SMSPS interventions. The teams supported 282 caregivers and 145 people living with HIV, followed up with 36 out of 41 recoveries (87.8%) in the community and 15 out of 15 children in daycare, and provided support to 14

Machine Translated by Google

Of the 17 separated and orphaned children in households (82.4%), 8 out of 13 (61.5%) were reintegrated, and 52 results were announced, including 16 positive ones; 77 children are being followed up.

- **In Haut-Uélé,** 4 new confirmed cases, 23 confirmed cases under follow-up, 7 new suspected cases and 26 suspected cases under follow-up benefited from SMSPS interventions in the CTEs of Isiro, Pawa, Boma Mangbetu and Wamba; the teams supported 58 accompanying persons, 80 PPLs (including 25 members of the national coordination), 10 children separated in the community and 10 household members, supported 3 EDS and announced 25 results, including 4 positive ones; 25 unloading kits were given to the recovered patients of the CTE in Isiro.

- **In Bas-Uélé,** 5 PPL, 6 household and community members and 1 contact were supported, 4 discharged cured are being followed up, 1 reluctance was lifted and 3 results were announced (all negative).

2.8.2. Challenges

- **In Ituri, only** 14 out of 28 affected health zones are covered (50.0%) and the Mandima health zone has no SMSPS coverage, which requires strengthening the capacities of psychologists and APS/TPS to ensure interventions around cases.

- **In Bas-Uélé,** SMSPS coverage remains embryonic, with activities having only just begun in the Viadana ZS.

- **In Haut-Uélé,** the mobility of community health workers remains difficult in Wamba and Boma Mangbetu, nutritional kits are lacking for weaned infants in Wamba and Isiro, the community health worker lacks computer equipment, and data collection remains weak in Boma Mangbetu and Pawa

- **In Tshopo,** there is a need to train the APS of the ZS of Wanie-Rukula and Bafwasende and a shortage of communication units and motorcycles for the movement of the pillar.

2.9. Logistics
2.9.1. Main actions.
Ensure the mobility of all field teams in all affected provinces.

2.10. Security
2.11. Main actions
- **In North Kivu,** EDS teams were escorted for the burial of 2 confirmed cases at the Bundji public cemetery and 6 confirmed cases from CT Matanda and Katwa at the Manghera cemetery, as well as for a community death in the Kitulu neighborhood (Makisi health district), and IPC teams for the decontamination of the Emmanuel Dalson dispensary (Tamende health district) and a household in the Ngongolyo health district

2.12. PSEA (Prevention of Sexual Exploitation and Abuse)
2.12.1. Main actions
- **In Ituri,** 54 people (24 women, 12 men, 16 girls, including 1 person living with HIV, and 2 boys) were sensitized on the PSEAH in the Bunia health zone (motorcyclists from the Wewa parking lot, breastfeeding women, staff and customers of a hair salon), and the display of messages and the distribution of leaflets continued in Bunia, Aru, Mongbwalu, Mambasa and Fataki. Training of trainers on the PSEAH started in the Bunia health zone with 27 participants (19 women and 8 men), and training sessions brought together 61 teachers and members of parent committees in Komanda (23 women and 38 men) as well as 22 members of COPA, teachers, community leaders and women's associations in Tchomia (8 women and 14 men); Awareness campaigns reached 3,502 students and staff from five schools in Aru, 22 community and traditional leaders in Lita, and 8 painters in Bunia. Seventy-two staff members were briefed and signed the code of conduct. The display of messages and the distribution of leaflets continued in Bunia, Aru, Mongbwalu, Mambasa, and Fataki.

- **In North Kivu,** 20 PSEA/GBV focal point volunteers (15 women and 5 men) were briefed in Beni with the support of the ICRC, and 118 people were sensitized on PSEA and reporting channels (495555 and 151) in Beni (53, including 20 members of ECUMR), Butembo (11) and Katwa (54), with the distribution of 15 leaflets.

Machine Translated by Google

• **In Bas-Uélé,** 67 RECOs (28 women and 39 men) from the AS Bambili and Agri-Uélé (ZS of Ganga) and 84 community members (36 women and 48 men) from schools and churches in Buta were sensitized on EAS in the context of the MVE.

• **In Haut-Uélé,** 23 PPLs and caregivers (6 men and 17 women) were sensitized at CH Gamba (Isiro), where 13 codes of good conduct were signed, as well as 30 media actors from Isiro (24 men and 6 women) and 40 community members at CS CECCA 16 in Wamba (17 men and 23 women); 6 out of 7 health zones (85.7%) know the reporting mechanism.

2.12.2. Challenges

At the national level, 4 out of 7 DPS (57.1%) reported their PSEA activities (Ituri, North Kivu, Haut-Uélé and Bas-Uélé); in Ituri, only 13 out of 36 ZS (36.1%) know the reporting mechanism, only one partner (WHO) reports its activities and awareness materials and computer kits are lacking; in North Kivu, the mapping of community complaint management mechanisms is not mastered; in Haut-Uélé and Bas-Uélé, means of transport and awareness materials remain insufficient.

MESSAGES CLES

ÿ In the provinces of Ituri, North Kivu and Haut-Uélé, it is observed active community transmission while sporadic notification is observed in Tshopo province.

2. In the last 24 hours.

ÿ Over the past 24 hours, all suspected cases (100.0%) have been investigated; performance is consistent with data reported by the provinces. However, contact tracing remains inconsistent (83.5% vs. 80.0%) and warrants close attention to prevent uncontrolled chains of transmission.

Maintain a level of vigilance regarding epidemiological surveillance aspects, consolidate IPC and CREC actions at the community level while providing quality care in dedicated care structures with adequate reception capacities.

TIMELINE OF KEY EVENTS

ÿ From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.

ÿ Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.

Machine Translated by Google

TASK FORCE PRESIDENTIAL EBOLA 17

Ministry of Public Health, Hygiene and Social Welfare

National Institute of Public Health

Public Health Emergency Operations Center

MVE17 Incident Management System

Director General of INSP

Dr. MWAMBA KAZADI Dieudonné

Email: dieudonne.mwamba@insp.cd

Incident Manager

Professor Dr. AHUKA MUNDEKE Steve

Email: steve.ahuka@unikin.ac.cd

Operations Manager

Dr. KOBBE NGWAMA John

Editorial Team

Professor Dr. MULANGU KATULUMBA Félix-Médard

Dr. KATUSI BISELENGE Jules

Mrs. WISE DIE Marie

Dr. DIUR MANANASHI Floryan

Mr. Kamuina Kebela John

Contact :

www.insp.cd

contact@sgiebola17.org

Freephone number: 151. www.sante.gouv.cd